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Obsessive-Compulsive Disorder (OCD)

Obsessive-compulsive disorder (OCD) is a mental health condition in which you have frequent unwanted thoughts that cause you to perform repetitive behaviors. Treatment for OCD usually involves psychotherapy and medication. The sooner OCD is diagnosed and treated, the better the outlook.

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Overview



Learn what OCD is, what can cause it and the treatment options.

What is obsessive-compulsive disorder (OCD)?

Obsessive-compulsive disorder (OCD) is a condition in which you have frequent unwanted thoughts and sensations (obsessions) that cause you to perform repetitive behaviors (compulsions). The repetitive behaviors can significantly interfere with social interactions and performing daily tasks.

OCD is usually a life-long (chronic) condition, but symptoms can come and go over time.

Everyone experiences obsessions and compulsions at some point. For example, it's common to occasionally double-check the stove or the locks. People also often use the phrases "obsessing" and "obsessed" very casually in everyday conversations. But OCD is more extreme. It can take up hours of a person's day. It gets in the way of normal life and activities. Obsessions in OCD are unwanted, and people with OCD don't enjoy performing compulsive behaviors.

What is the difference between OCD and OCPD?

Even though they sound similar, obsessive-compulsive disorder (OCD) and [obsessive-compulsive personality disorder \(OCPD\)](#) are different conditions.

OCPD is a [personality disorder](#) that causes an extensive preoccupation with perfectionism, organization and control.

People with OCD are usually aware that their obsessions and compulsions are problematic and accept that they need professional help to treat the condition. People with OCPD usually don't think there's anything wrong with their behavior and beliefs.

Who does OCD affect?

OCD can affect anyone. The average age of onset is 19 years. About 50% of people with OCD begin to have symptoms in childhood and adolescence.

It's rare for someone to develop OCD after the age of 40.

How common is obsessive-compulsive disorder (OCD)?

Obsessive-compulsive disorder is relatively common. It affects 1.6% to 2.3% of the general U.S. population.

Symptoms and Causes

Obsessive-Compulsive Disorder (OCD)

A condition in which you have frequent unwanted obsessions that cause you to perform compulsions.

Obsessive symptoms can include:



Fear of germs or dirt.



Fear of making a mistake.



Need for order, neatness, symmetry or perfection.



Fear of causing harm to yourself or someone else.

Compulsive symptoms can include:





OCD is usually a life-long condition, but symptoms can come and go over time.

What are the symptoms of OCD?

The main symptoms of OCD are obsessions and compulsions that interfere with normal activities. For example, symptoms may often prevent you from getting to work on time. Or you may have trouble getting ready for bed in a reasonable amount of time.

You may know that these symptoms are problematic, but you can't stop them. Symptoms of OCD may come and go, ease over time or worsen over time.

If you or your child have symptoms of OCD that interfere with your/their daily life, you should talk to a healthcare provider.

Obsessions in OCD

In OCD, obsessions are unwanted, [intrusive thoughts](#) or mental images that cause intense anxiety. People with OCD can't control these thoughts. Most people with OCD realize that these thoughts are illogical or irrational.

Common examples include:

- Fear of coming into contact with perceived contaminated substances, such as germs or dirt.
- Fear of causing harm to yourself or someone else because you're not careful enough or you're going to act on a violent impulse.

- Unwanted thoughts or mental images related to sex.
- Fear of making a mistake.
- Excessive concern with morality ("right or wrong").
- Feelings of doubt or disgust.
- Need for order, neatness, symmetry or perfection.
- Need for constant reassurance.

Compulsions in OCD

In OCD, compulsions are repetitive actions that you feel like you have to do to ease or get rid of the obsessions.

People with OCD don't want to perform these compulsive behaviors and don't get pleasure from them. But they feel like they have to perform them or their anxiety will get worse. Compulsions only help temporarily, though. The obsessions soon come back, triggering a return to the compulsions.

Compulsions are time-consuming and get in the way of important activities that you value. They don't have to match the content of your obsessions.

Examples include:

- Arranging things in a very specific way, such as items on your dresser.
- Bathing, cleaning or washing your hands over and over.
- Collecting or hoarding items that have no personal or financial value.
- Repeatedly checking things, such as locks, switches and doors.
- Constantly checking that you haven't caused someone harm.
- Constantly seeking reassurance.
- Rituals related to numbers, such as counting, doing a task a specific amount of times, or excessively preferring or avoiding certain numbers.
- Saying certain words or prayers while doing unrelated tasks.

Compulsions can also include avoiding situations that trigger obsessions. One example is refusing to shake hands or touch objects that other people touch a lot, like doorknobs.

What causes OCD?

Researchers don't know what exactly causes OCD. But they think several factors contribute to its development, including:

- **Genetics:** Studies show that people who have a first-degree relative (biological parent or sibling) with OCD are at a higher risk for developing the condition. The risk increases if the relative developed OCD as a child or teen.

- **Brain changes:** Imaging studies have shown differences in the frontal cortex and subcortical structures of the brain in people who have OCD. OCD is also associated with other neurological conditions that affect similar areas of your brain, including [Parkinson's disease](#), [Tourette's syndrome](#) and [epilepsy](#).
- **PANDAS syndrome:** PANDAS is short for "pediatric autoimmune neuropsychiatric disorders associated with streptococcal infections." It describes a group of conditions that can affect children who have had strep infections, such as [strep throat](#) or [scarlet fever](#). OCD is one of these conditions.
- **Childhood trauma:** Some studies show an association between childhood trauma, such as [abuse](#) or neglect, and the development of OCD.

Diagnosis and Tests

How is OCD diagnosed?

There's no test for OCD. A healthcare provider makes the diagnosis after asking you about your symptoms and medical and mental health history. Providers use criteria explained in the [Diagnostic and Statistical Manual of Mental Disorders, 5th Edition \(DSM-V\)](#) to diagnose OCD.

The criteria include:

- Having obsessions, compulsions or both.
- The obsessions or compulsions take up a lot of time (more than an hour per day).
- The obsessions or compulsions cause distress or affect your participation in social activities, work responsibilities or other life events.
- The symptoms aren't caused by substances, alcohol, medications or another medical condition.
- The symptoms aren't explained by a different mental health condition, such as [generalized anxiety disorder](#), [eating disorder](#) or [body image disorder](#).

Management and Treatment

How is OCD treated?

The most common treatment plan for OCD involves [psychotherapy](#) (talk therapy) and medication.

If this treatment doesn't help your OCD symptoms and your symptoms are severe, your provider may recommend [transcranial magnetic stimulation \(TMS\)](#).

Psychotherapy for OCD

Psychotherapy, also called talk therapy, is a term for a variety of treatment techniques that aim to help you identify and change unhealthy emotions, thoughts and behaviors. You work with a mental health professional, such as a [psychologist](#).

There are several types of psychotherapy. The most common and effective forms for treating OCD include:

- [Cognitive behavioral therapy \(CBT\)](#): During CBT, a therapist will help you examine and understand your thoughts and emotions. Over several sessions, CBT can help alter harmful thoughts and stop negative habits, perhaps replacing them with healthier ways to cope.
- [Exposure and response prevention \(ERP\)](#): ERP is a type of CBT. During ERP, a therapist exposes you to your feared situations or images and has you resist the urge to perform a compulsion. For example, your therapist may ask you to touch dirty objects but then stop you from washing your hands. By staying in a feared situation without anything negative happening, you learn that your anxious thoughts are just thoughts and not necessarily reality.
- [Acceptance and commitment therapy \(ACT\)](#): ACT helps you learn to accept obsessive thoughts as just thoughts, taking the power away from them. An ACT therapist will help you learn to live a meaningful life despite your OCD symptoms.

Mindfulness techniques such as [meditation](#) and relaxation can also help with symptoms.

Medication for OCD

Medications called serotonin reuptake inhibitors (SRIs), selective SRIs (SSRIs) and tricyclic antidepressants may help treat OCD.

Healthcare providers most often recommend SSRIs for OCD and prescribe them at much higher doses than they do for anxiety or depression. U.S. Food and Drug Administration (FDA)-approved SSRIs include:

- [Fluoxetine](#).
- [Fluvoxamine](#).
- [Paroxetine](#).
- [Sertraline](#).

It may take up to eight to 12 weeks for these medications to start working.



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Outlook / Prognosis

What is the prognosis of OCD?

The prognosis (outlook) of OCD can vary. OCD is often a lifelong condition that can wax and wane.

People with OCD who receive appropriate treatment often experience increased quality of life and improved social, school and/or work functioning.

If you don't receive treatment, the cycle of obsessions and compulsions is more difficult to break and treat, as structural changes in your brain take place. Because of this, it's key to seek medical care as soon as possible if you or your child experience symptoms.

Prevention

Can I prevent OCD?

You can't prevent OCD. But early diagnosis and treatment can help reduce its symptoms and its effects on your life.

Living With

How do I take care of myself if I have OCD?

Aside from seeking medical treatment for OCD, practicing self-care can help manage your symptoms. Examples include:

- Getting quality sleep.
- Exercising regularly.
- Eating a healthy diet.
- Spending time with loved ones who support you and understand OCD.
- Practicing relaxation techniques, such as meditation, yoga, massage and visualization.
- Joining an in-person or online support group for people who have OCD.

When should I see my healthcare provider about OCD?

If you or your child have received an obsessive-compulsive disorder diagnosis, you'll likely need to see your healthcare provider and/or mental health professional regularly to make sure your treatment is working.

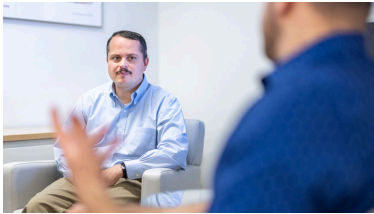
If you're taking medication for OCD and notice any unpleasant side effects, talk to your healthcare provider.

A note from Cleveland Clinic

It's important to remember that obsessive-compulsive disorder (OCD) is a mental health condition. As with all mental health conditions, seeking help as soon as symptoms appear can help decrease the disruptions to your life. Healthcare providers and mental health professionals can offer treatment plans that can help you manage your obsessions and compulsions.

Care at Cleveland Clinic

Your mental well-being is just as important as your physical well-being. Cleveland Clinic's mental health experts can help you live life to the fullest.



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